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Abstinence Violation Effect

Susan E. Collins¹ and Katie Witkiewitz²

¹Department of Psychiatry and Behavioral Sciences, University of Washington, Harborview Medical Center, Seattle, WA, USA

²University of New Mexico, Albuquerque, New Mexico, USA

Synonyms

AVE

Definition

The abstinence violation effect (AVE) refers to the negative cognitive (i.e., internal, stable, uncontrollable attributions; cognitive dissonance) and affective responses (i.e., guilt, shame) experienced by an individual after a return to substance use following a period of self-imposed abstinence from substances (Curry, Marlatt, & Gordon, 1987).

Description

AVE in the Context of the Relapse Process

The AVE was introduced into the substance abuse literature within the context of the “relapse process” (Marlatt & Gordon, 1985, p. 37). Relapse has been variously defined, depending on theoretical orientation, treatment goals, cultural context, and target substance (Miller, 1996; White, 2007). It is, however, most commonly used to refer to a resumption of substance-use behavior after a period of abstinence from substances (Miller, 1996). The term *relapse* may be used to describe a prolonged return to substance use, whereas *lapse* may be used to describe discrete, circumscribed “slips” during sustained abstinence (Marlatt & Gordon, 1985, p. 32).

As originally described by Marlatt and Gordon (1985), the relapse process typically begins when a person who has achieved abstinence encounters a

situation that puts them at high risk for relapse (i.e., a high-risk situation). If the person is able to cope effectively with the high-risk situation, they may experience increased self-efficacy (i.e., confidence to avoid a lapse). If, on the other hand, they are unable to cope with the high-risk situation, they may experience decreased self-efficacy. If this decreased self-efficacy is paired with positive outcome expectancies for substance use, a person may have a heightened risk for a lapse. If a lapse occurs, it may be experienced as a “violation” of self-imposed abstinence, which gave rise to the term, AVE. The AVE may, in turn, precipitate a relapse if the person turns to substances repeatedly to cope with the resulting negative cognitive and affective reactions of the AVE.

AVE: Cognitive and Affective Responses to a Lapse

The AVE is characterized by a lapse paired with a specific constellation of negative cognitive and affective reactions. The role of cognitions stems from attributional theory (Weiner, 1974): a person might attribute their lapse to factors that are internal, global, and uncontrollable. For example, people may believe the lapse occurred due to their own, irreparable character defects or chronic disease determinants. The associated affective component stems from dissonance between the lapse and one’s perceived self-image as an abstainer, which together with the attributions, can lead to feelings of guilt, shame, and hopelessness (Marlatt & Gordon, 1985). People who experience the AVE are more likely to progress from a lapse to a relapse (Miller, Westerberg, Harris, & Tonigan, 1996), and several studies have demonstrated the role of the AVE in predicting relapse among drinkers (Collins & Lapp, 1991), smokers (Curry et al., 1987), dieters (Mooney, Burling, Hartman, & Brenner-Liss, 1992), and marijuana users (Stephens, Curtin, Simpson, & Roffman, 1994).

In contrast, if people attribute the lapse to external, unstable (i.e., changeable), and controllable causes, they may not interpret the lapse as a threat to their self-image and may instead view it as a unique occurrence that can be avoided in the future. This attributional style may diffuse the

person's affective response to the lapse and reduces the likelihood of a progression from lapse to relapse (Laws, 1995; Marlatt & Gordon, 1985; Walton, Castro, & Barrington, 1994). Averting the AVE may have lasting effects: as the situation is less affectively charged, the individual might be open to exploring the determinants of the lapse and to experimenting with alternative coping strategies in the future. This may, in turn, lead to increased self-efficacy and more effective coping across various high-risk situations (Marlatt & Gordon, 1985).

Preventing the AVE Response

Clinicians may help clients interrupt the relapse process at various points and ultimately avoid the AVE. First, clinicians can help clients identify and apply effective behavioral and cognitive strategies in high-risk situations to avoid the initial lapse altogether. If a lapse occurs, clinicians should be empathetic and nonjudgmental in their approach (Miller & Rollnick, 2002) and should help clients reframe the lapse as the product of multiple factors (versus only internal factors), as being controllable (versus uncontrollable), and as situation-specific (versus global; Larimer, Palmer, & Marlatt, 1999). A step-by-step exploration may help clients learn how to interrupt the relapse process at various points to avoid future lapses, the AVE and/or relapses (Larimer et al., 1999). Further, the clinician may elicit and positively reinforce clients' existing coping skills to support the clients' self-efficacy and may teach clients additional behavioral and cognitive coping strategies for application in future high-risk situations, as necessary (Witkiewitz & Marlatt, 2007). Finally, clinicians should assess whether clients are coping adequately with the negative affective component of the AVE, which may otherwise precipitate future lapses or relapses.

Cross-References

- Addictive Behaviors
- Alcohol Abuse and Dependence
- Binge Drinking

- Health Risk (Behavior)
- National Institute on Alcohol Abuse and Alcoholism
- Relapse, Relapse Prevention

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